

Mucosal and Cutaneous Lesions

The mouth is the most frequent site of involvement in patients with cicatricial pemphigoid and is often the first—and sometimes the only—site affected. Lesions commonly involve the gingiva, buccal mucosa, and palate; other sites such as the alveolar ridge, tongue, and lips are also susceptible. A frequent oral manifestation is **desquamative gingivitis**. Other lesions may present as tense blisters that rupture easily or as mucosal erosions resulting from epithelial fragility. Oral lesions can lead to a delicate white pattern of reticulated scarring. In severe cases, adhesions may form between the buccal mucosa and alveolar process, around the uvula and tonsillar fossae, and between the tongue and floor of the mouth. Gingival involvement can result in tissue loss and dental complications, including caries, periodontal ligament damage, and loss of bone mass and teeth.

Ocular involvement is common in cicatricial pemphigoid and may become sight-threatening. Ocular lesions typically begin as conjunctivitis that progresses insidiously to scarring. Early disease is often subtle and nonspecific, usually starting unilaterally before becoming bilateral over several years. Patients may report burning, dryness, or a foreign-body sensation in one or both eyes; frank blisters on conjunctival surfaces are rarely observed. Early ocular changes are best detected by slit-lamp examination. Because lesions may be localized to the upper tarsal conjunctiva, they can be missed without eyelid eversion. Chronic ocular disease leads to scarring characterized by shortened fornices, **symblepharons** (fibrous adhesions between bulbar and palpebral conjunctival surfaces), and, in severe cases, **ankyloblepharon** (fibrous fusion of the upper and lower eyelids).

A few skin lesions on the upper body are also sometimes noted. Associated symptoms are site-specific, as detailed later.

Major Cicatricial Pemphigoid Autoantigens

AUTOANTIGEN	MW (kD)	LOCATION SSS/ULTRA	PASSIVE TRANSFER STUDIES
BPAG1	230	Epid/HD	
BPAG2	180	Epid/HD-af	Experimental IgG against the NC16A domain of BPAG2 induces subepidermal blisters in newborn mice resembling those in bullous pemphigoid.
Integrin $\beta 4$	~205	Epid/HD-af	
Integrin $\alpha 6$	~120	Epid/HD-af	
Laminin 5	400– 440	Derm/LL-LD interface	Experimental IgG against laminin 5 induces subepidermal blisters in newborn and adult mice resembling those in anti-laminin 5 cicatricial pemphigoid. Patient IgG causes similar blisters in human skin grafts on immunodeficient adult mice.
Type VII collagen	290	Derm/AF	Both experimental and patient IgG against type VII collagen induce subepidermal blistering in animal models.